

Writing Task 1 Doctors

Read the cases notes below and complete the writing task that follows

Time allowed: 40 minutes

Today's Date

16.02.09

Patient History

Miss Cathy Jones - 25 year old single woman
Occupation - receptionist
Family history of deep vein thrombosis
On progesterone-only pill (POP) for contraception
No previous pregnancies

15.02.09

Subjective

Presents to GP surgery at 7 pm, after work
Complains of lower abdominal pain since the evening before, worse in right iliac fossa
Unsure of last menstrual period, has had irregular bleeding since starting
POP 2 months ago, New partner for past 2 months
No bladder or bowel symptoms

Objective

Mild right iliac fossa tenderness, no rebound / guarding

Apyrexial, pulse 88, BP 110/70
Vaginal examination - quite tender in right fornix. No masses

Assessment

Non-specific abdo pain

Plan: Asks her to return in morning for blood test and reassessment

16.02.09

Subjective

Pain has worsened overnight. Now severe constant pain.
Some slight vaginal bleeding overnight also.
Felt faint while waiting in reception.
On questioning, has left shoulder-tip pain also.

Objective

Very tender in the right iliac fossa, with guarding and rebound tenderness
Apyrexial, Pulse 96, BP 110/70
On vaginal examination, has cervical excitation and markedly tender in the right fornix.
Pregnancy test result positive
Urine dipstick clear

Assessment

Suspected ectopic pregnancy

Plan: You ring the on duty Gynaecology Registrar and ask for urgent assessment, and are instructed to send her to the A&E Department with a referral letter.

Writing Task

You are the GP, Dr Sally Brown. Write Referral letter to the Gynaecology Registrar at the Mater Hospital, South Brisbane. Ask to be kept informed of the outcome.

In your answer:

- Expand the relevant case notes into complete sentences
- Do not use note form
- The body of the letter should not be more than 200 words
- Use correct letter format

Task 1 Model Letter: Cathy Jones

Gynaecology Registrar
A&E Department
Mater Hospital
South Brisbane

Dear Doctor,

Re: Cathy Jones DOB 1.12.83

Thank you for seeing this 25 year old woman who I suspect has an ectopic pregnancy.

This is her first pregnancy. She presented to the surgery yesterday evening with vague lower abdominal pain. She started the progesterone-only pill for contraception two months ago, when she started a new relationship, and has had some irregular bleeding since then. Therefore she is unsure of her exact last menstrual period. Yesterday, she was mildly tender only and her observations were normal.

However, on review this morning her pain had worsened overnight, she is very tender in the right iliac fossa, with rebound and guarding, and on vaginal examination there is cervical excitation, and marked tenderness in the right fornix. Her pregnancy test is positive.

I am concerned that she may have an ectopic pregnancy, and would appreciate your urgent assessment.

Please keep me informed of the outcome.

Yours sincerely,

Dr Sally Brown (GP)

Task 2 Case Notes: Brendan Cross

Time allowed: 40 minutes

Read the case notes below and complete the writing task that follows:

Today's Date

21/01/10

Patient History

Brendan, 8 years old boy

Has a sister 6 years, brother 3 years

Mother – housewife

Father – Naval Officer currently on active duty in Indonesia

P.M.H- NAD

Brendan is on 50th percentile for height & weight

Allergy to nuts – hospitalised with anaphylaxis 2 years ago following exposure to peanuts

14/01/10

Subjective

Fever, sore throat, lethargy, many crying spells – all for 3 days.

Objective

Temperature - 39.8°C

Enlarged tonsils with exudate

Enlarged cervical L.N.

Ab - NL

CVS – NL

RR – NL

Probable Diagnosis

Tonsillitis (bacterial)

Management

Oral Penicillin 250mg 6/h, 7days

Paracetamol as required.

Review after 5days if no improvement.

19/1/10

Subjective

Mother concerned – sleepless nights, difficulty coping with husband away – mother-in-law coming to help.

Brendan not eating complaining of fever, right knee joint pain, tiredness, lethargy – for 2 days

Objective

Temperature - 39.2°C

Hypertrophied tonsils

Cervical lymph node – NL

Swollen R. Knee Joint

No effusion

Mid systolic murmur, RR - normal

Investigation

ECG, FBC, ASOT ordered

Treatment

Brufen 100mg tds, review in 2 days with investigation reports

21/1/10

No change of symptoms

ECG – prolonged P-R interval

ESR – increased

ASOT – Increased

Diagnosis

? Rheumatic fever

Plan

Contact Mater Paediatric Centre to arrange an urgent appointment with Dr Alison Grey, Paediatric Consultant requesting further investigation and treatment.

Writing Task

You are GP, Dr Joseph Watkins, Greenslopes Medical Clinic, 294 Logan Rd, Greenslopes, Brisbane 4122. Write a referral letter to Dr Alison Grey, Mater Paediatric

Centre, Vulture Street, Brisbane 4101.

Do not use note form. The body of your letter should be approximately 200 words. Use correct letter format.

Task 2 Model Letter: Brendan Cross

21/01/2010

Dr. Joseph Watkins
Greenslopes Medical Clinic
294 Logan Rd
Greenslopes
Brisbane, 4122

Dr. Alison Grey
Mater Paediatric Centre
Vulture Street,
Brisbane, 4101

Dear Dr. Grey,
Re: Brendan Cross

Thank you for seeing this 8 year old boy who has demonstrated features consistent with rheumatic fever. His developmental and past medical history were unremarkable except for an allergy to peanuts . His mother has difficulty in caring for both his illness and two other small children with his father being away due to his work as a naval officer

He presented with symptoms suggestive of acute bacterial tonsillitis on 14/01/10, when fever and sore throat had occurred over the previous 3 days, associated with lethargy and crying spells. High temperature (39.8), enlarged tonsils with exudate and cervical lymphadenopathy were found. Therefore oral penicillin and paracetamol were prescribed.

Regrettably, he returned on 19/01/10 with worsening symptoms. Fever had persisted with right knee joint pain. Also, he appeared restless, and was finding it difficult to eat and sleep. Examination revealed hypertrophied tonsils and a swollen right knee joint without signs of effusion. There was mid-systolic murmur on heart auscultation. Brufen was prescribed but was not effective. Today, blood tests results reported elevated erythrocyte sedimentation rate and anti-streptolysin O titre. An abnormal electrocardiogram indicated prolonged P-R interval.

I consider Brendan needs admission for further investigation and stabilization. I would appreciate your urgent attention to his condition.

Yours sincerely,

Dr. Watkins

Word Count 204 words

Task 3 Case Notes: David Taylor

Time allowed: 40 minutes

Read the cases notes below and complete the writing task that follows:

Today's Date

07/11/10

Patient History

Mr David Taylor, 38 years old, married, 3 children

Landscape Gardener

Runs own business.

No personal injury insurance

Active, enjoys sports

Drinks 1-2 beers a day. More on weekends.

Smokes 20-30 cigarettes/day

P.M.H-

Left Inguinal Hernia Operation 2008

12/08/10

Subjective

C/o left knee joint pain and swelling, difficulty in strengthening the leg.

Has history of twisting L/K joint 6 months ago in a game of tennis.

At that time the joint was painful and swollen and responded to pain killers.

Finds injury is inhibiting his ability to work productively.

Worried as needs regular income to support family and home repayments.

Objective

Has limp, slightly swollen L/K joint, tender spot on medial aspect of the joint and no effusion.

Temperature- normal

BP 120/80

Pulse rate -78/min

Investigation - X ray knee joint

Management

Voltarin 50 mg bid for 1/52

Advise to reduce smoking

Review if no improvement.

25/8/10

Subjective

Had experienced intermittent attacks of pain and swelling of the L/K joint

No fever

Unable to complete all aspects of his work and as a result income reduced

Reduced smoking 15/day

Objective

Swelling +

No effusion

Tender on the inner-aspect of the L/K joint

Flexion, extension – normal

Impaired range of power - passive & active

Diagnosis ? Injury of medial cartilage

Investigation – ordered MRI

Management

Voltarin 50mg bid for 1 week

Review after 1 week with investigations

07/11/10

Subjective

Limp still present

Patient anxious as has been unable to maintain full time work.

Desperate to resolve the problem

Weight increase of 5kg

Objective

Pain decreased, swelling – no change

No new complications

MRI report – damaged medial cartilage

Management Plan

Refer to an orthopaedic surgeon, Dr James Brown to remove damaged cartilage in order to prevent future osteoporosis. You have contacted Dr Brown's receptionist and you have

arranged an appointment for Mr Taylor at 8am on 21/11/10

Writing Task

You are the GP, Dr Peter Perfect. Write a referral letter to Orthopaedic Surgeon, Dr. James Brown: 1238 Gympie Road, Chermside, 4352. In your letter expand the relevant case notes into complete sentences. Do not use note form. The body of your letter should be approximately 200 words. Use correct letter format.

Model Letter 3: Damaged Cartilage

07/11/10

Dr. James Brown
1238 Gympie Rd
Chermside, 4352

Dear Dr. Brown,

Re: David Taylor

Thank you for seeing this patient, a 38-year-old male who has a damaged cartilage in the left knee joint. He is self-employed as a landscape gardener, and is married with 3 children.

Mr. Taylor first presented on 12 August 2010 complaining of pain and swelling in the left knee joint associated with difficulty in strengthening the joint. He initially twisted this joint in a game of tennis 6 months previously, experiencing pain and swelling which had responded to painkillers. Examination revealed a slightly swollen joint and there was a tender spot in the medial aspect of the joint. Voltarin 50mg twice daily was prescribed.

Despite this treatment, he developed intermittent pain and swelling of the joint. The x-ray showed no evidence of osteoarthritis. However, the range and power including passive and active movements was impaired. An MRI scan was therefore ordered and revealed a damaged medial cartilage.

Today, the pain was mild but the swelling has not reduced. Mr Taylor is keen to resolve the issue as it is affecting his ability to work and support his family.

In view of the above I believe he needs an arthroscopy to remove the damaged cartilage to prevent osteoarthritis in the future.

Yours sincerely,

Dr. Peter Perfect
Word Count: 200 words

Writing Task 4 – Karen and William Conway

Time allowed: 40 minutes

Read the cases notes below and complete the writing task that follows:

Today's Date: 15/03/10

Patient History

Mrs Karen Conway has consulted you, her GP, as she and her husband have been trying to conceive for about 18 months without success, and she is becoming concerned that there may be something wrong.

Karen is a 32 year old solicitor.

Her husband, William, is a 33 year old accountant.

Karen: previous pregnancy 10 years ago, terminated. William does not know about this.

William: no previous pregnancies.

15/02/10

Subjective

Karen attends on her own. She reports that neither she or William have any significant medical problems. Neither partner smokes, although she reports that William drinks quite heavily. Also he has to travel regularly with his job.

Married for 3 years, and decided to try for a pregnancy in May 2006, when Karen stopped the pill. Was on Microgynon 30 for the previous 5 years.

Periods are regular

No history of gynaecological problems, or sexually transmitted diseases.

Objective

Karen overweight BMI 28.

Pulse and BP normal.

Abdo exam normal.

As is some time since she last had a smear test, you do a vaginal examination, which is normal, and take a cervical smear.

Assessment

Although the couple have only been trying to conceive for 18 months, Karen is clearly very anxious, and so you decide that further investigation is appropriate.

Plan

Blood tests for Karen required to confirm that her hormone levels are normal and that she

is ovulating. You explain to Karen that it is necessary for you to see her husband, William also, and ask her to make an appointment for him. Karen anxious that you do not reveal her history of a termination of pregnancy to him.

15/03/10

Karen re-attends, accompanied by her husband William Conway.

Subjective

Karen's baseline blood tests are normal, except the test for ovulation is borderline. However Karen informs you that she has used a home ovulation-prediction test which did show positive, so it is likely that she is ovulating. Smear test result negative. As Karen reported, William has no significant medical problems. He says he only drinks 10 units per week, which does not agree with Karen's previous comments that he drinks heavily. He also explains that he works away from home approximately 2 weeks out of 4, so he is not so concerned that Karen has not conceived yet, as he thinks that it is because they haven't been trying long enough. Therefore not keen on being investigated.

Objective

William refuses to be examined as he doesn't think there is a problem.

Assessment

Karen is even more anxious that when first seen and wants to be referred to an infertility specialist, whereas William is quite reluctant. She tells you that her sister has recently had IVF treatment.

Plan

You suggest that William do a semen analysis, to which he agrees reluctantly, under pressure from Karen. You try to reassure Karen that it is not unusual to take up to 2 years to conceive, and there are no obvious risk factors, however at Karen's insistence, you agree to refer them to a specialist, while awaiting the results of the semen analysis. You give them some general advice regarding timing of intercourse, and suggest to Karen that she should try to lose some weight. Lastly you check that Karen is taking folic acid, 400 micrograms daily.

Writing Task

You are her GP, Dr Claire Black. Write the referral letter to Dr John Expert MBBS FRANZCOG, Gynaecologist and IVF Specialist, St Mary's Infertility Centre, Wickham Terrace, Brisbane.

In your letter expand the relevant case notes into complete sentences. Do not use note form. The body of your letter should be approximately 200 words. Use correct letter format.

Task 4 Model Letter

15.3.10

Dr John Expert, MBBS, FRANZCOG
St Mary's Infertility Centre
Wickham Terrace
Brisbane

Dear Dr Expert,

Re: Karen Conway, DOB 1.2.78
William Conway, DOB 2.1.77

This couple have requested referral as they have been trying to conceive for approximately 18 months without success. I have tried to reassure them that there is no reason to be concerned yet, particularly as William works away from home regularly and there are no risk factors in their history, however Karen, particularly, was anxious to be referred sooner rather than later.

Karen has regular periods and has no history of gynaecological problems or sexually transmitted diseases. Her hormone tests are all normal, and ovulation confirmed. I did a smear test on 15.2.08 which was negative, and examination then was normal. She is a little overweight, with a Body Mass Index of 28, and I have advised that she lose some weight. Karen is taking folic acid 400 mcg daily.

William also has no significant medical problems and he declined examination. However, he has agreed to do a semen analysis, but I don't as yet have the results. I will forward them on in due course.

Thank you for seeing them and continuing with investigations as you think appropriate. I do wish them success.

Yours sincerely,

Dr Claire Black (GP)

Word length: 184 words

Writing Task 5 Alison Martin

Time allowed: 40 minutes

Read the case notes below and complete the writing task that follows:

Today's Date

10/02/10

Patient History

Alison Martin

Female

28 year old, teacher.

Patient in your clinic for 10 years

Has 2 children, 4 years old and 10 months old, both pregnancies and deliveries were normal. Husband, 30 yr old, manager of a travel agency. Living with husband's parents.

Has a F/H of schizophrenia, symptoms controlled by Risperidone

Smoking-nil

Alcohol- nil

Use of recreational drugs – nil

09/01/10

Subjective

c/o poor health, tiredness, low grade temperature, unmotivated at work, not enjoying her work. No stress, loss of appetite or weight.

Objective

Appearance- nearly normal

Mood – not depressed

BP- 120/80

Pulse- 80/min

Ab, CVS, RS, CNS- normal

Management

Advised to relax, start regular exercise, and maintain a temperature chart. If not happy follow up visit required

20/01/10

Subjective

Previous symptoms – no change

Has poor concentration and attention to job activities, finding living with husband's parents difficult. Says her mother-in-law thinks she is lazy and is turning her husband against her. Too tired to do much with her children, mother-in-law takes over. Feels anxiety, poor sleep, frequent headaches.

Objective

Mood- mildly depressed

Little eye contact

Speech- normal

Physical examination normal

Tentative diagnosis

Early depression or schizophrenia

Management plan

Relaxation therapy, counselling

Need to talk to the husband at next visit

Prescribed Diazepam 10mg/nocte and paracetamol as required

Review in 2/52

10/02/10

Subjective

Accompanied by husband and he said that she tries to avoid eye contact with other people, reduced speech output, impaired planning, some visual hallucinations and delusions for 5 days

Objective

Mood – depressed

Little eye contact

Speech – disorganised

Behaviour- bizarre

BP 120/80

Pulse- 80

Ab, CVS, RS, CNS- normal

Probable diagnosis

Schizophrenia and associated disorders

Management plan

Refer to psychiatrist for assessment and further management.

Writing Task

You are the GP, Dr Ivan Henjak. Write a referral letter to Psychiatrist, Dr. Peta Cassimatis: 1414 Logan Rd, Mt Gravatt, 4222. In your letter expand the relevant case notes into complete sentences. Do not use note form. The body of your letter should be approximately 200 words. Use correct letter format.

Task 5 Model Letter: Alison Martin

10/02/10

Dr. Peta Cassimatis
1414 Logan Rd,
Mt Gravatt, 4222

Re: Alison Martin

Dear Doctor,

I am writing to refer Mrs. Martin, a 28-year-old married woman, who is presenting with symptoms suggestive of schizophrenia.

Mrs Martin has been a patient at my clinic for the last 10 years and has a family history of schizophrenia. She is a teacher with two children, aged 4 years and 10 months, and lives with her husband's parents.

She first presented at my clinic on 9 January 2010 complaining of tiredness, a lack of motivation at work and a low grade fever. On review after ten days, she did not show any improvement. She displayed symptoms of paranoia and was suffering from poor sleep, anxiety and frequent headaches. In addition, she was mildly depressed with little eye contact. Relaxation therapy and counselling were started and Diazepam 10 mg at night was prescribed based on my provisional diagnosis of early depression or schizophrenia.

She presented today accompanied by her husband in a depressed state, showing little eye contact, bizarre behaviour and disorganised speech. Despite my management, her symptoms have continued to worsen with a 5-day history of reduced speech output, impaired planning ability as well as some visual hallucinations and delusions.

In view of the above, I would appreciate your attention to this patient.

Yours sincerely,

Dr. Ivan Henjak

Word Count: 204 words